

Write your **birth plan** using broad headings—see the box below for **some ideas**. Make the plan as **detailed** or as **vague** as you want. Read about the birth and all the options available to help you think ahead to what you might want. It's important to make sure you feel **confident** and **fully informed** about all of your choices beforehand.

» **Cesarean sections:** Who you want with you in the operating room; what music (if any) you want playing there; whether you would like the screen lowered at the moment your baby is born so that you can see the obstetrician lift the baby out; if you would like silence among the medical staff as your baby is lifted out so that yours can be the first voice your baby hears; who you would like to hold your baby if you have to have general anesthesia, and whether you are OK for your birth partner to tell anyone else about the birth and baby before you've woken up from the anesthesia. Also, if you've had general anesthesia, whether you'd like your baby to be bottle-fed from the breast milk bank, or to have formula if he needs to be fed before you wake up. Whether you would like a video or photographs taken of your baby as he's lifted out of your uterus; whether you'd like to have skin-to-skin contact and to try to breast-feed while your uterus and abdomen are being sewn up, or you're fine with your partner holding your baby until you're settled in the post-op room.

Q My partner has some ideas for the birth plan, but I don't agree with them all. Should I try to fit them in?

Your birth plan is about your care during your labor, so ultimately it should reflect your wishes. However, parenting is something that you are going to do together, so if there are things he feels strongly about that matter less to you, then in the spirit of this new and exciting joint venture, make sure you give his views consideration. When it comes to anything that might happen to your body your wishes and the advice of your doctor always come first.

Q Will I need to have my birth plan with me while I'm in labor? What if I'm too distracted to remember what's in it?

Your doctor will put a copy of your birth plan in your records so that it is available to whoever is caring for you at the time of your labor. It's also a good idea to give a copy to your birth partner, who can represent your wishes if you aren't able to articulate them yourself. Make sure to brief your birth partner, though, that although you might have said you want to avoid an assisted birth, for example, if the situation dictates that this is the safest way for your baby to be born and you understand why, your doctor's advice is paramount and so the birth plan would change.

Q What does VBAC stand for and is it something I should consider?

VBAC stands for "vaginal birth after cesarean section" and it means exactly what it says: if you had a C-section for your last birth, you might be able to have a vaginal birth instead this time. Assuming that you are able to have a VBAC, whether or not you want one is entirely

a matter of personal choice and practicality. For example, you will recover more quickly from a vaginal birth than from a C-section, which can make it easier to take care of your other children with a newborn in tow, too. A VBAC can have fewer complications than a second C-section, but this is not always the case.

Q Are there any reasons that I wouldn't be able to have a VBAC?

You will be able to have a VBAC as long as your previous cesarean section was straightforward, and for pregnancy rather than anatomical reasons. So, if you had a complication during labor that made C-section a safer way to deliver your baby, there's no reason why you wouldn't be able to have a VBAC this time. However, if you weren't able to give birth vaginally because, for example, you have a small pelvis that makes it hard for your baby to get through, you will probably have to have a second C-section. If the cut in your uterus was a standard horizontal incision, you should be fine for a VBAC, but cuts elsewhere into your uterus will need to be considered on a case-by-case basis.

Q What's the likelihood that I'll end up having another C-section, even if I start out with the hope of a VBAC?

Assuming you had a straightforward, nonanatomical-related previous cesarean section (see above), there is a 70 to 85 percent success rate for VBAC. Specifically relating to your previous C-section, the main obstacle to success is that there's around a 0.5 percent chance that you develop a tear along your scar line during your contractions. The risk of a tear occurring during VBAC rises to 1 percent if you have had two previous C-sections; there is little data on the risks after three or more C-sections. It's often a case of waiting to see what happens.

70–85% Most hospitals record VBAC success rates of 70 percent to 85 percent.